



MINISTRY OF LABOUR AND
SOCIAL PROTECTION

STATE DEPARTMENT FOR SOCIAL
PROTECTION AND SENIOR CITIZEN AFFAIRS

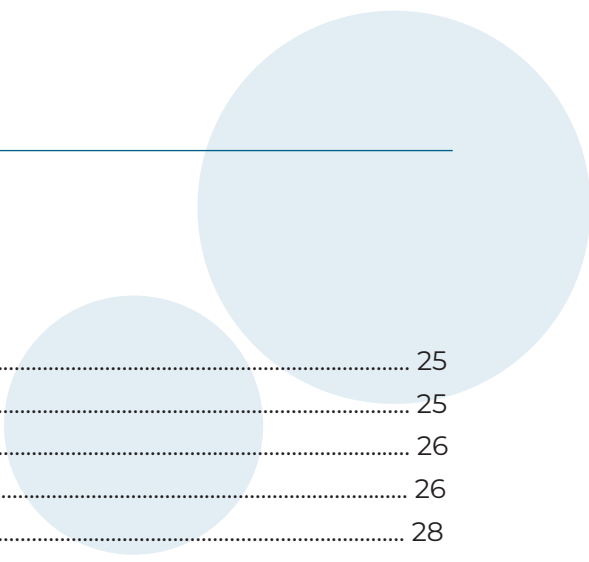
Community Based
Inclusive Development

AWARENESS BOOKLET

KENYA
VISION 2030

TABLE OF CONTENTS

LIST OF ACRONYMS	IV
DEFINITION OF TERMS	V
FOREWORD	VII
ACKNOWLEDGMENTS	VIII
EXECUTIVE SUMMARY	IX
CHAPTER ONE: INTRODUCTION TO CBID	1
EVOLUTION OF CBID	2
PURPOSE OF BOOKLET	3
USERS OF THE BOOKLET	3
PRINCIPLES OF CBID	3
IMPORTANCE OF COMMUNITY BASED INCLUSIVE DEVELOPMENT	4
OBJECTIVES OF CBID	4
CHAPTER TWO: LEGAL AND POLICY PROVISIONS	5
GLOBAL CONTEXT	5
REGIONAL CONTEXT	8
NATIONAL CONTEXT	8
POLICY FRAMEWORKS	11
CHAPTER THREE: COMMUNITY BASED INCLUSIVE DEVELOPMENT CBID COMPONENTS	12
THE CBID MODEL	13
HEALTH COMPONENT	14
KEY CONCEPTS IN CBID AND HEALTH	14
GOAL OF CBID IN HEALTH	16
ROLE OF CBID IN HEALTH	16
EDUCATION COMPONENT	16
KEY CONCEPTS IN CBID AND EDUCATION	17
GOAL OF CBID IN EDUCATION	18
ROLE OF CBID IN EDUCATION	18
FORMS OF EDUCATION	19
LIVELIHOOD COMPONENT	21
GOAL OF CBID IN LIVELIHOOD	22
ROLE OF CBID IN LIVELIHOOD	22
ELEMENTS OF THE LIVELIHOOD COMPONENT	23



SOCIAL COMPONENT	25
KEY CONCEPTS OF THE SOCIAL COMPONENT	25
GOAL OF CBID IN SOCIAL	26
ROLE OF CBID IN SOCIAL	26
EMPOWERMENT COMPONENT	28
KEY CONCEPTS OF THE EMPOWERMENT CONCEPT	29
GOAL OF CBID IN EMPOWERMENT	29
ROLE OF CBID IN EMPOWERMENT	29
CHAPTER FOUR: COMMUNITY BASED INCLUSIVE DEVELOPMENT SERVICES AND SUPPORT PROGRAMMES	31
STATE-LED PROGRAMMES AND KEY ROLES	32
NON-STATE ACTORS AND DEVELOPMENT PARTNERS	34

LIST OF ACRONYMS

AGPO	Access to Government Procurement Opportunities
APDK	Association for the Physically Disabled of Kenya
CBID	Community Based Inclusive Development
CBM	Christian BlindMission
COK	Constitution of Kenya
CT-OVC	Cash Transfer for Orphans and Vulnerable Children
HSNP	Hunger Safety Net Programme
HIV	Human Immunodeficiency Virus
AIDS	Acquired Immunodeficiency Syndrome
ICT	Information and Communication Technology
MDAs	Ministries, Departments and Agencies
NCPWD	National Council for Persons with Disabilities
NFDK	National Fund for the Disabled of Kenya
OPCT	Orphans and Vulnerable Persons Cash Transfer
OPDs	Organizations of and for Persons with Disabilities
PWSD-CT	Persons with Severe Disabilities Cash Transfer
SDGs	Sustainable Development Goals
TAF	The Action Foundation
UDPK	United Disabled Persons of Kenya
UHC	Universal Health Care
UN	United Nations
UNCRPD	United Nations Convention on the Rights of Persons with Disabilities
UNESCO	United Nations Educational, Scientific and Cultural Organization
VRCs	Vocational Rehabilitation Centres
WHO	World Health Organization

DEFINITION OF TERMS

Awareness creation: this takes into consideration persons with disabilities, families, and Communities on the empowerment opportunities.

Capacity-building: is the process of developing and strengthening skills, abilities processes and resources for persons with disabilities to achieve the goal of meaningful participation.

Education Assessment: is a wide variety of methods or tools used to evaluate, measure and document the academic readiness, learning progress, skill acquisition or educational needs of learners and trainees.

Educational Assessment Resource Centers: are set up for the educational assessment of children and their placement to appropriate education services.

Extra-curricular activities: refer to non-academic programs that support personal development, creativity, social interaction, and inclusion especially for learners with disabilities. These are activities outside the regular classroom that help build confidence, skills, and friendships.

Home Based Education: is the education of school aged children at home or a variety of other places other than physical schooling.

Inclusion: the process of accessibility focuses on adjusting home, school, and society so that all the individuals, regardless of their differences, can have equal opportunity, participation to interact, play, learn, work and experience the feeling of belonging, and experiment to develop in accordance with their potentials and difficulties.

Mainstreaming: a strategy through which concerns, needs and experiences of persons with disabilities are made an integral part or dimension of the design, budgetary allocation, implementation, monitoring, and evaluation, and reporting of policies and programmes in all political, economic, and societal spheres so that persons with disabilities benefit equally and inequality is not perpetuated.

Meaningful Participation: Active and equal involvement of persons with disabilities in all aspects of the society where their views, ideas and recommendations are taken into consideration. For this to happen it would be necessary to ensure reasonable accommodation is provided.

Participation: Being a contributor brings social recognition and is important in the empowerment process. Persons with disabilities can participate and make positive contributions at many different levels.

Political participation: Promoting the participation of persons with disabilities in politics is an important approach to empowerment.

Reasonable Accommodation: necessary and appropriate modification and adjustments not imposing a disproportionate or undue burden, where needed in a particular case, to ensure to persons with disabilities the enjoyment or exercise on an equal basis with others of all human rights and fundamental freedoms.

Inclusive education: The process of addressing and responding to the diversity of needs of all learners through increasing meaningful participation in learning, cultures and communities, and eliminating within and from education.

Resource Room: A separate, remedial classroom in an inclusive school where learners and trainees with disabilities are given occasional direct, specialized instruction and academic remediation and assistance as individuals or in groups.

Rehabilitation: the process of enabling individuals to achieve and maintain maximum independence and full participation in all aspects of life

Self-Advocacy: This refers to Persons with disabilities having a voice of their own. It means knowing about their rights and responsibilities, speaking up for their rights, and being able to make choices and decisions about their lives.

Self-employment: is a term applied to economic activities in both the formal and informal economies which are owned, operated and managed by an individual or a group

FOREWORD



It gives me great pleasure to present this Community-Based Inclusive Development (CBID) Awareness Booklet, developed against the backdrop of Kenya's commitment to advancing the rights of persons with disabilities and recognizing the broad impact of disability across all sectors of society children, youth, women, and older persons.

The Government of Kenya acknowledges the need for a Community-Based Rehabilitation approach as a strategy to uphold the rights, dignity, and protection of persons with disabilities. This booklet reflects ongoing developments in disability inclusion and emphasizes that persons with disabilities possess valuable skills, knowledge, and experiences that can drive national development. Promoting inclusion is therefore essential.

The CBID booklet addresses challenges faced by persons with disabilities and provides a framework for strategies that enable meaningful participation in development processes. Its goal is to promote inclusion and participation of persons with disabilities in all societal activities toward achieving national development targets, recognizing them as integral members of the population whose rights must be respected.

This booklet represents the government's commitment to providing training and creating an environment conducive to the enjoyment of freedoms, liberties, and opportunities for persons with disabilities. Successful implementation of CBID requires collaboration among stakeholders, supported by instruments such as the Convention on the Rights of Persons with Disabilities (2008), the Constitution of Kenya (2010), and the Persons with Disabilities Act (2025), which together create an enabling environment for full inclusion.

DR. ALFRED N. MUTUA, EGH
CABINET SECRETARY
MINISTRY OF LABOUR AND SOCIAL PROTECTION

ACKNOWLEDGEMENTS



The State Department for Social Protection and Senior Citizens Affairs acknowledges the collaborative effort toward the development of this booklet. The objective of this booklet is to ensure that Ministries, Departments and Agencies actively participate towards Community Based Rehabilitation to ensure that all persons with disabilities participate at the community level and taking into consideration the barriers that face them.

Special gratitude goes to the technical working committee from both State and Non state actors for the crucial role played towards finalization of this booklet including Ministries, Departments, State Agencies and Organization of and for Persons with Disabilities.

I would like to acknowledge with much appreciation the crucial role played by officers from the State Department for Social Protection and Senior Citizen Affairs drawn from the Directorate of Social Development and the National Council for Persons with Disabilities who have been instrumental in spearheading the finalization of this document. I also recognize with gratitude the support provided by partners such as United Disabled Persons of Kenya, The Action Foundation, Christian Blind Mission, Kenya Association for the Intellectually Handicapped and all actors in the social protection sector.

Lastly, I take cognizance of the crucial role by Christian Blind Mission and The Action Foundation from inception to completion of this booklet for their financial support. Together we move forward in building a more inclusive, empowered and supportive communities.

A handwritten signature in blue ink, appearing to read 'Joseph M. Motari', written over a light blue circular graphic element.

JOSEPH M. MOTARI, CBS
PRINCIPAL SECRETARY
STATE DEPARTMENT FOR SOCIAL PROTECTION AND SENIOR CITIZEN AFFAIRS

EXECUTIVE SUMMARY

The Community Based Inclusive Development Awareness Booklet recognizes the challenges and barriers in relation to disability and is organized in five chapters.

Chapter one provides the definition of Community Based Inclusive Development objectives, evolution, principles and importance of CBID.

Chapter two highlights the institutional framework and implementation framework, as well as the coordination structure and the roles of state and non-state actors and the Organizations of and for Persons with Disabilities.

CBID booklet is broad based, and its implementation will involve wide sector cooperation amongst Persons with disabilities, Public Private Partnerships, communities, and development partners. It is anticipated that the implementation of CBID will enhance inclusion and disability mainstreaming in decision making and development processes.

CHAPTER 1

According to Global Report on health equity for persons with disabilities 2022, 16% of the world population is estimated to be persons with disabilities. An earlier report by world bank and WHO 2011 indicated that Persons with disabilities constitute 20% of the world's poorest people, with the number of women with disabilities being higher than that of men. Further , 80% of persons with disabilities live in developing countries.

The institutionalization of disability mainstreaming in the public sector through the Public Service Reforms and Results Based Management initiatives in Kenya has ensured a higher visibility of disability matters . At the beginning of 2009/2010 financial year, disability mainstreaming was made a key target for performance contracting and in all sector policies and programmes whereby all Ministries, Departments and Agencies (MDAs) were expected to mainstream disability in their sector plans and programmes.

In order to do so the government has adopted community based inclusion Development approach on which this booklet is about. The government efforts, complemented by other interventions by different stakeholders have been very instrumental in getting disability to be at the core of National Development aimed at ensuring that persons with disabilities live in dignity and enjoy their human rights and barriers that exclude them from full and equal participation in society are addressed

EVOLUTION OF CBID

Community-based rehabilitation (CBR) was initiated by the World Health Organization (WHO) following the Declaration of Alma-Ata in 1978. It was promoted as a strategy to improve access to rehabilitation services for people with disabilities in low-income and middle-income countries, by making optimum use of local resources. Over the past 30 years through collaboration with other UN organizations, non-governmental organizations and organization of persons with disabilities.

In 2010 the World Health Organization developed CBR guidelines to provide CBR managers, among others, with practical suggestions on how to develop or strengthen CBR programmes and ensure that people with disabilities and their family members are able to access the benefits of the health, education, livelihood and social sectors. The guidelines have a strong focus on empowerment through facilitation of the inclusion and participation of persons with disabilities, their family members, and communities in all development and decision-making processes. The guidelines encourage CBR programmes to be evaluated and further research to be carried out on the effectiveness and efficiency of CBR in diverse contexts.

CBR has evolved into a multi-sectoral, inclusive framework and rights-based approach for community development targeting and involving persons with disabilities, their families, and their respective organizations as primary stakeholders.

As a strategy, CBID seeks to further the inclusion and meaningful participation of persons with disabilities in society by:

- o Removing the barriers to development that persons with disabilities face;
- o Delivering quality services and programmes.
- o Addressing the causes of disability.
- o Bringing persons with and without disabilities together on an equal basis.
- o In achieving these aims, CBID helps to reduce poverty and improve the lives of everyone in the community.

The CBR matrix provides a common framework for CBID programmes. CBID programmes are not expected to implement every component and element of the CBR matrix. CBID Programmes are designed based on local needs, priorities, and resources. In addition, CBID programmes develop partnerships/ alliances to ensure that persons with disabilities and their family members can benefit from all the components and elements of the CBR Matrix. Persons with disabilities are also made aware of their own responsibilities and role as they participate in the community.

PURPOSE OF BOOKLET

This booklet is designed to promote awareness and understanding of disability inclusion through Community-Based Inclusive Development (CBID). It emphasizes practical strategies for empowering persons with disabilities and fostering equal participation in society. The content is grounded in the principles of the WHO 2010 Community-Based Rehabilitation (CBR). By aligning with these global standards, the booklet aims to guide stakeholders in creating inclusive communities that uphold dignity, rights, and opportunities for all.

USERS OF THE BOOKLET

This booklet is intended for a wide range of users including:

- o The Public.
- o Social Development Officers.
- o Organizations of/ for persons with disabilities.
- o Persons with disabilities.
- o Social Development Resource Persons.
- o Society /community.
- o Non-government organizations.
- o Ministries, Departments and Agencies.
- o Development agencies.
- o National Government Administration Officers.

PRINCIPLES OF CBID

The principles of CBID form fundamental guidelines that ensure programs are inclusive, empowering and community driven. These principles have been drawn from WHO and aligned with the UNCRPD that adopt the human rights approach to development. The human rights model takes universal human rights as a starting point. Persons with disabilities are seen to have a right to access all within their society on an equal basis with others.

Disability-inclusive development takes a rights-based approach that incorporates social model thinking where external barriers are identified in conjunction with the person with a disability being the focal point in the attainment of their rights. The rights-based approach adopts awareness, participation, comprehensive accessibility and twin track as core disability-inclusive development principles. The twin track approach encourages mainstream inclusion working alongside disability specific initiatives.

The principles include;

- o Respect for inherent dignity, individual autonomy including the freedom to make one's own choices, and independence of persons.
- o Non-discrimination.
- o Respect for difference and acceptance of persons with disabilities as part of human diversity and humanity.
- o Equality of opportunity.
- o Equality between men and women.
- o Respect for the evolving capacities of children with disabilities and respect for the rights of children with disabilities to preserve their identities.

IMPORTANCE OF COMMUNITY BASED INCLUSIVE DEVELOPMENT

According to the WHO (2010), Community Based Inclusive Development is important due to the following reasons:

- o It improves the lives of persons with disabilities through addressing the barriers to accessing development.
- o It brings awareness to the community of the diverse needs for persons with disabilities.
- o It creates a positive attitude.
- o It improves understanding of persons with disabilities and their rights.
- o It brings about equity.
- o It promotes inclusion of persons with disabilities at the community governance, allocation for decision making.

OBJECTIVES OF CBID

1. To maximize physical and mental capacities of persons with disabilities.
2. Removing barriers and making services accessible to persons with disabilities to enable their meaningful participation.
3. To protect, promote and advocate for the rights of persons with disabilities.
4. To create awareness on the participation of persons with disabilities in development programs.

CHAPTER 2

This chapter covers legal and policy provisions on disability based on international, regional and national contexts.

GLOBAL CONTEXT

The Universal Declaration of Human Rights was proclaimed by the United Nations General Assembly (UNGA) of 1948. Under this declaration, “all human beings are born free and equal in dignity and rights.” The 1975 UN Declaration on the Rights of Persons with disabilities expanded the space for persons with disabilities. It borrowed heavily from the Bill of Rights requiring that persons with disabilities be accorded with respect, opportunity for rehabilitation, education, employment, human dignity and enjoyment of life within a family set up.

The global awareness created during the UN International Year for Disabled Persons --IYDP in 1981 expanded social participation and equality for disabled persons. This was followed by the 1982/1992 UN Decade for Persons with Disabilities. To ensure the decade had the desired impact, a comprehensive document entitled World Programme of Action Concerning Disabled Persons was developed and adopted through a UN resolution in 1982. The document provided guidelines on effective measures for the realization of full participation of persons with disabilities in social life, development and equality. UN agencies were encouraged to globally implement the document in accordance with their areas of specialization.

The Vocational Rehabilitation and Employment (Disabled Persons) Convention, 1983, ensures that appropriate vocational rehabilitation measures are made available to all categories of disabled persons. It also promotes the employment of disabled persons in the open labour market.

A panel of experts conducted a mid-term review of the UN Decade for Persons with Disabilities and concluded that the World Programme of Action was not achieving the expected outcome and that something more binding was necessary. However, there was no consensus on what additional measures were required. The UN and interested parties such as persons with disabilities thus embarked on a search for consensus, which resulted in the development of the UN Standard Rules on Equalization of Opportunities for Persons with Disabilities.

This document, with twenty-two rules on the behaviour of states, was the most comprehensive ever. The Rules were divided into four categories. The first category included four rules, which address preconditions for equalization of opportunities. These include awareness raising, medical care and rehabilitation and support services. The second and most important covered Rules 5 to 12 on target areas of equalization of opportunities. These are accessibility, education, employment, income maintenance and social security, family life and personal integrity, culture, religion, recreation and sports. The last 10 rules were on measures of implementation and mechanisms for monitoring. The document was presented to the UN Assembly and adopted in December 1993.

Although the UN Standard Rules were guidelines that were not binding to Governments, the level of awareness built around them had greater influence than the World Programme of Action. During the early years after their adoption, a lot of organizations of persons with disabilities were established in many parts of the world. More disability legislations and policies were put in place in more countries.

Seven years after adoption of the UN Standard Rules, it became clear that goodwill was not enough to change the lives of persons with disabilities. More pressure through enforceable instruments was required to impact heavily on countries to provide a favorable social environment. Efforts to increase cooperation, integration and awareness on disability issues by governments and relevant organizations remained insufficient in promoting full and effective participation and equal opportunities for persons with disabilities in economic, social, cultural and political life. There was still a need for a more comprehensive and binding instrument to promote and protect the rights and dignity of persons with disabilities.

The idea of a convention was once again floated and through intense lobbying an ad hoc committee with membership from governments, Non-governmental Organisations (NGOs) and DPOs was set up in 2001 to work on a draft international convention.

United Nations Convention on the Rights of Persons with disabilities (UNCRPD): It was a product of a long history culminating in nearly five years (between 2002 and 2006) negotiations by the United Nations Ad Hoc committee which included among others member states and the disability organizations. The convention which is the first human rights instrument in this century and also the very first one specifically dedicated to human rights of persons with disabilities was adopted by the United Nations General Assembly on December 13th, 2006, and the process of signing and ratification began in March 2007. The 20th instrument of ratification which was required before the treaty could come into effect was deposited on 3rd April 2008 and the treaty therefore came into effect as from 3rd May 2008.

Kenya signed the CRPD in 2007 and ratified it in 2008. The purpose of the Convention is to promote, protect and ensure the full and equal enjoyment of all human rights and fundamental freedoms by all Persons with disabilities and to promote respect for their inherent dignity. Specific articles most relevant to CBID components include, Article 8 (Awareness-raising), Article 12 (Equal recognition before the law), Article 13 (Access to justice), Article 19 (Living independently and being included in the community), Article 23 (Respect for home and the family) , Article 24 (Education), Article 27 (Work and employment), Article 28 (Adequate standard of living and social protection) , Article 29 (Participation in political and public life), Article 30 (Participation in cultural life, recreation, leisure and sport)

Sustainable Development Goals (SDGs): The SDGS adopted in September 2015 by the general assembly succeeded the Millennium Development goals (2000-2015) which had no reference to persons with disabilities. The 2030 agenda for Sustainable Development includes 17 goals built on the principle of 'Leave no one behind'. The agenda holds a deep promise for persons with disabilities everywhere. All the 17 goals are relevant for the inclusion agenda of development of persons with disabilities; however, the following make specific reference to disability.

Goal 1 (End poverty in all its forms everywhere) 4 (Ensure inclusive and equitable quality education and promote lifelong learning opportunities for all), 8 (Promote sustained, inclusive and sustainable economic growth, full and productive employment and decent work for all), Goal 10.2 (By 2030, empower and promote the social, economic and political inclusion of all, irrespective of age, sex, disability, race, ethnicity, origin, religion or economic or other status), 11(Make cities and human settlements inclusive, safe, resilient and sustainable) and 17 (Strengthen the means of implementation and revitalize the Global Partnership for Sustainable Development).

REGIONAL CONTEXT

Protocol to the African Charter on Human Rights and Peoples Rights on the Rights of Persons with disabilities in Africa: The African Disability protocol was adopted by the African Union (AU) assembly on 29th January 2018. Kenya ratified the protocol in February 2022. The primary objective of the Protocol is to promote, safeguard and ensure the complete and equal exercise of all human and people's rights for individuals with disabilities in Africa, as well as to ensure respect for their inherent dignity. The protocol complements the UN Convention on the Rights of Persons with disabilities (UNCRPD) by addressing the rights of persons with disabilities from an African perspective, considering the lived realities of individuals with disabilities on the continent while maintaining the core values and principles as set forth in the UNCRPD.

The following articles in the protocol are relevant to CBID; Article 5 (Non-discrimination), Article 7 (Equal recognition before the law), Article 13 (Right to access justice), Article 14 (Right to live in a community), Article 16 (Right to education), Article 17 (Right to health), Article 18 (Habilitation and Rehabilitation), Article 19 (Right to work), Article 22 (Self representation), Article 23 (Right to freedom of expression and opinion), Article 24 (Right to information).

NATIONAL CONTEXT

Constitution of Kenya (2010): The Constitution of Kenya, 2010, confers upon its citizens, including Persons with Disabilities, the right to the enjoyment of all human rights and fundamental freedoms. The basis of these protections is spelt out in the national values and principles of governance. The following tabulated Articles are relevant to disability inclusion:

ARTICLE	CONTENT
7(3)(b)	Promotes development and use of indigenous languages, Kenya Sign Language, Braille, and other communication formats and technologies accessible to persons with disabilities.
10	Binds all State organs, State officers, public officers, and all persons applying or interpreting the Constitution to make or implement public policy decisions that among other things promote non-discrimination. It also seeks to minimize barriers to equalization of opportunities for persons with disabilities in all aspects including socio-cultural, economic, and political life.
11	Culture (1) This Constitution recognizes culture as the foundation of the nation and as the cumulative civilization of the Kenyan people and nation. (2) The State shall (a) promote all forms of national and cultural expression through literature, the arts, traditional celebrations, science, communication, information, mass media, publications, libraries and other cultural heritage.
27	Further guarantees the right to equality and freedom from discrimination and expressly prohibits discrimination on the grounds of disability.

41	Labour relations (1) Every person has the right to fair labour practices.
43	Economic and social rights (1) Every person has the right— (a) to the highest attainable standard of health, which includes the right to health care services, including reproductive health care; (b) to accessible and adequate housing, and to reasonable standards of sanitation; (c) to be free from hunger, and to have adequate food of acceptable quality; (d) to clean and safe water in adequate quantities; (e) to social security; and (f) to education.
45	Family (1) The family is the natural and fundamental unit of society and the necessary basis of social order and shall enjoy the recognition and protection of the State. (2) Every adult has the right to marry a person of the opposite sex, based on the free consent of the parties. (3) Parties to a marriage are entitled to equal rights at the time of the marriage, during the marriage and at the dissolution of the marriage. (4) Parliament shall enact legislation that recognizes - (a) marriages concluded under any tradition, or system of religious, personal or family law; and (b) any system of personal and family law under any tradition, or adhered to by persons professing a particular religion, to the extent that any such marriages or systems of law are consistent with this Constitution.
48	Access to justice The State shall ensure access to justice for all persons and, if any fee is required, it shall be reasonable and shall not impede access to justice.
53(1)	Children (1) Every child has the right - (a) to a name and nationality from birth; (b) to free and compulsory basic education; (c) to basic nutrition, shelter and health care; (d) to be protected from abuse, neglect, harmful cultural practices, all forms of violence, inhuman treatment and punishment, and hazardous or exploitative labour; (e) to parental care and protection, which includes equal responsibility of the mother and father to provide for the child, whether they are married to each other or not; and (f) not to be detained, except as a measure of last resort, and when detained, to be held— (i) for the shortest appropriate period of time; and (ii) separate from adults and in conditions that take account of the child's sex and age.
53(2)	A child's best interests are of paramount importance in every matter concerning the child.
54	Outlines the entitlements to persons with disabilities, a. To be treated with dignity and respect and to be addressed and referred to in a manner that is not demeaning, b. To access educational institutions and facilities for persons with disabilities that are integrated into society to the extent compatible with the interests of the person, c. To reasonable access to all places, public transport, and information; to use Sign language, Braille, or other appropriate means of communication; d. To access materials and devices to overcome constraints arising from the person's disability. The State shall ensure the progressive implementation of the principle that at least five percent of the members of the public in elective and appointive bodies are persons with disabilities.

Persons with Disabilities Act, 2025: This is a comprehensive law that promotes and protects the rights and equalization of opportunities for persons with disabilities. It covers rights, rehabilitation, and equal opportunities for Persons with disabilities and aligns with the Constitution of Kenya 2010 and UNCRPD. The act has various provisions, rights, and privileges relating to health, education, disability mainstreaming, awareness creation, and accessibility, among others. It establishes the National Council for Persons with disabilities (NCPWD) which among other role supervises the implementation of the Act by other relevant MDAs and County Government that promotes and protects equalizations of opportunity.

The Children's Act, 2022: It aims to safeguard the rights and welfare of children. It provides a robust legal framework for protecting children from abuse, neglect, and exploitation, ensuring their access to education, health, and essential services. The Act also establishes provisions for parental responsibility, alternative care, and the administration of children's services.

Basic Education Act, 2013: It mandates the establishment of special and integrated schools for learners with disabilities, ensuring they can access education in appropriate settings, whether separate or within mainstream society.

The Early Childhood Education Act, 2021: It guarantees all children, including those with disabilities, the right to early childhood education without discrimination. It requires county governments to provide accessible ECE, identify and assess children with disabilities, and ensure no actions hinder their access. These provisions promote inclusive and equitable education for every child.

The Technical and Vocational Education and Training Act, 2013: The TVET Act 2013 in Kenya promotes disability inclusion by requiring institutions to mainstream special needs, ensure accessibility (physical and instructional), provide reasonable accommodation, offer guidance/counselling, and implement policies that align with the Rights and Protection of Persons with Disabilities.

Employment Act, 2007: Contains elaborate provisions aimed at protecting persons from discrimination in employment which explicitly prohibits an employer from discriminating directly or indirectly against an employee or prospective employee or harassing an employee or prospective employee on grounds of disability.

Work, Injury, Benefit Act, 2007: Provides compensation for work-related injuries and diseases, including disability, through different benefit categories. The Act outlines the rights of employees to receive benefits in case of work-related incidents and establishes the framework for claims and compensation.

The Public Procurement and Asset Disposal Act, 2015: Provides for the national and county governments to facilitate affirmative action for disadvantaged groups in accordance with the Constitution and advance their participation in the procurement process. The Act supports Access to Government Procurement Opportunities (AGPO) which facilitates the youth, women, and persons with disabilities-owned enterprises to be able to participate in government procurement. It mandates at least 30% of all government procurement opportunities to be set aside for women, youth and persons with disabilities.

The Community Groups Registration Act, 2022: It provides a structure for the management of groups' registration, coordination, mobilization, and control to legitimize and sustain them.

Social Protection Act, 2025: It provides support for vulnerable groups including orphans, older persons, and persons with disabilities; cushion people from social and economic risks; and promote dignity and well-being. Through this Act persons with disabilities will receive respite services, rehabilitative services and psychosocial support among others.

Social Health Insurance Act, 2023: It covers persons with disabilities who are wholly dependent on and living with contributors. It expands benefits for persons with disabilities in terms of coverage for assistive devices, covers family wide protection, inclusion of long term and rehabilitation.

POLICY FRAME WORKS

Kenya Vision 2030 blueprint: Provides for specific strategies to improving access to all disadvantaged groups (e.g., Business opportunities, health, education, housing and justice) and minimizing vulnerabilities through prohibition of retrogressive practices (e.g., female genital mutilation and child labour) and by upscaling the training for person with disabilities and special needs. It also recognizes disability mainstreaming as one of the flagship projects under the Social Pillar.

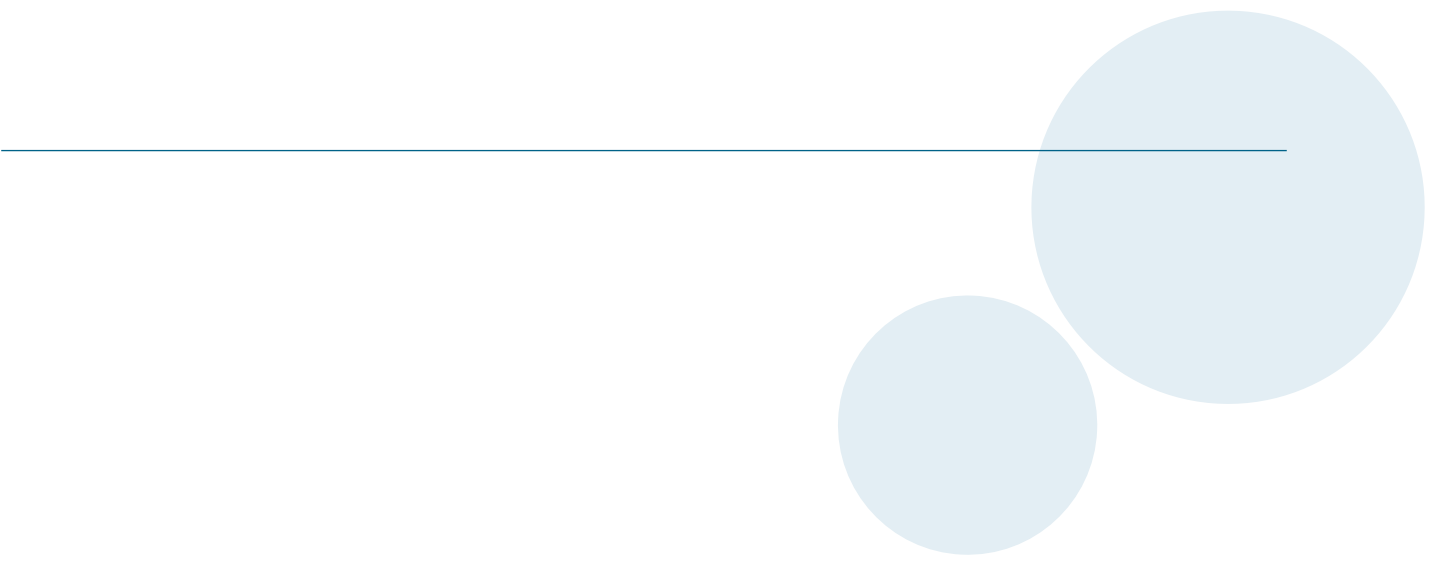
Persons with Disabilities National Policy 2024: The purpose of this policy is to inform and provide guidelines for planning budgeting, implementation, monitoring and evaluation of policies, legislations, programmes and specific measures for mainstreaming disability and empowering persons with disabilities.

Kenya Social Protection Policy 2024: The Policy presents the Government's vision of creating an all-inclusive society through the provision of sustainable mechanisms for the protection of persons living in situations of extreme poverty and vulnerability. The Policy identifies orphans and vulnerable children, persons with disabilities, older persons, the urban poor living in slum areas, people living in the streets in urban areas, people living with dilapidating illness, the internally displaced persons, among others as vulnerable and qualifying for social protection.

Sector Policy for Learners and Trainees with Disabilities: To provide a robust sector policy framework for provision of education and training for learners and trainees with disabilities.

National Policy on Gender and Development 2019: The overall goal of this policy is to achieve gender equality by creating a just society where women, men, boys and girls have equal access to opportunities in the political, economic, cultural, and social spheres of life."

Kenya Universal Health Coverage (UHC) Policy 2020-2030: "to ensure all Kenyans have access to essential quality health services without suffering financial hardship.



CHAPTER 3

Community Inclusive Based Development is a systematic approach that enables persons with disabilities to live fulfilling lives within their communities, using local resources and promoting equal participation. It is both a rehabilitation strategy and a community development model, focusing on equal opportunities, social inclusion, and empowerment. Persons with disabilities are encouraged to actively participate in community life.

CBID involves a variety of approaches to disability. Therefore, the World Health Organization (WHO) has developed a CBID framework, or matrix, which includes the various components of an effective CBID programme. These are health, education, livelihood, social, and empowerment. CBID is implemented through the combined efforts of persons with disabilities themselves, their families, organizations for persons with disabilities, the community, government, non-governmental organizations, and other relevant services such as health, education, vocational training, and social support. Its objectives are:

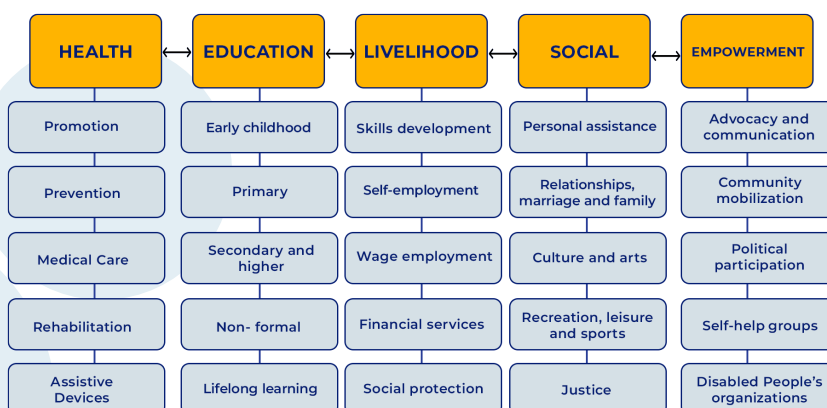
1. Maximize physical and mental ability
2. Ensure access to services and opportunities
3. Protect rights by removing barriers to participation

The enactment of the Persons with Disabilities Act 2025 in Kenya strengthens and gives a legal framework to the vision of CBID. The law aligns closely with the objectives and principles of CBID and the CRPD. It provides rehabilitation and habilitation services to help persons with disabilities develop their potential and mandates training of professionals in therapies and support services.

It guarantees inclusive education with individualized support, adapted curricula, assistive technologies, and county-level assessment centres. In employment, at least 5% of positions in national and county governments are reserved for persons with disabilities. Accessibility in public spaces, transport, ICT, and communication systems is mandatory under universal design principles. The Act enshrines non-discrimination, dignity, and equality before the law, with strict penalties for violations, including fines and imprisonment. Regulatory bodies can issue adjustment orders to compel institutions to make services and infrastructure accessible.

THE CBID MODEL

The CBID matrix provides a common framework for CBID programs. CBID programs are not expected to implement every component and element of the CBID matrix. CBID Programs are designed based on local needs, priorities and resources. In addition, CBID programs develop partnerships/ alliances to ensure that persons with disabilities and their family members can benefit from all the components and elements of the CBID Matrix (WHO, 2020).



HEALTH COMPONENT

Health is a basic human right, enshrined in the Constitution of Kenya (2010), Article 43 (1a): “Every person has the right to the highest attainable standard of health, which includes the right to health care services, including reproductive health care.” This right applies equally to persons with disabilities.

However, many persons with disabilities do not fully enjoy this right due to barriers such as poverty, distance to health facilities, lack of accessible transport, high medical costs, and limited access to health information. The World Health Organization (WHO) defines health as “a state of complete physical, mental, and social well-being, and not merely the absence of disease or infirmity.” Similarly, the Kenya Health Act (2017) upholds this holistic understanding of health. The Persons with Disabilities Act (2025) reinforces these rights by ensuring access to prevention, early identification, habitation, and rehabilitation services without discrimination.

CBID and health aim to ensure that persons with disabilities are not excluded from health services, but are supported at community level through inclusive, affordable, and participatory approaches. Institutionalization is only necessary when specialized care is required. By strengthening health promotion, prevention, medical care, rehabilitation, and assistive technology, CBID enables persons with disabilities to live healthier, more dignified, and fulfilled lives.

CBID programs complement government efforts by ensuring persons with disabilities achieve their highest attainable standard of health through five focus areas:

1. Health promotion
2. Prevention
3. Medical care
4. Rehabilitation
5. Assistive devices

KEY CONCEPTS IN CBID AND HEALTH

Health Promotion: It involves empowering individuals and communities to adopt healthy behaviours and prevent diseases. For persons with disabilities, this includes:

- o Inclusion in public health programs (health education, counselling, nutrition, physical activity, HIV prevention, substance abuse reduction).
- o Training health workers on disability-inclusive practices (e.g., sign language, braille).
- o Ensuring accessibility of health facilities in terms of affordability, infrastructure, and proximity.
- o Promoting disability-friendly communication channels such as Kenya Sign Language, braille, and other accessible formats.
- o Health promotion focuses not only on preventing illness but also on creating supportive environments for well-being.

Prevention: reduces the occurrence or impact of health conditions. It operates at three levels:

- **Primary prevention:** Avoiding health problems before they occur (e.g., immunization, good nutrition, safe environments).
- **Secondary prevention:** Early detection and treatment (e.g., screenings for cancer, cataract checks, antibiotics for infections).
- **Tertiary prevention:** Limiting or reversing the impact of established conditions through rehabilitation.

Examples include prenatal care to prevent birth-related complications, immunization to prevent polio, and routine health checks to detect disability early.

Medical Care: ensures early identification, treatment, and management of health conditions at different levels:

- **Primary care:** First contact services such as clinics and dispensaries.
- **Secondary care:** Specialized services at hospitals, often accessed by referral.
- **Tertiary care:** Highly specialized treatment in referral hospitals (e.g., surgery, cancer care).
- **Emergency care:** Immediate treatment for life-threatening conditions.
- **Palliative/rehabilitative care:** Long-term support to improve quality of life.

Rehabilitation and Habilitation: These are a set of measures that help persons with disabilities achieve and maintain optimal functioning in interaction with their environment. It is a series of interventions designed to help persons with disabilities attain and sustain optimal functioning in their daily environments. It applies to both those who acquire disabilities later in life and those who are born with them.

- **Rehabilitation:** Helps persons with disabilities regain lost skills and independence after illness or injury (e.g., physiotherapy after a fracture, speech therapy after a stroke).
- **Habilitation:** Supports children and individuals who have never developed certain skills due to congenital or early-onset disabilities (e.g., teaching a child with hearing loss to use sign language).

Both are essential for enabling persons with disabilities to participate fully in family, community, and economic life.

Assistive Devices and Technology: These are a range of products, equipment and technology which assists persons with disabilities including technology to perform a particular task. Assistive devices are tools that enhance independence and participation. Examples include:

- **Mobility devices:** Walking sticks, wheelchairs.
- **Prostheses and orthoses:** Artificial limbs, splints.
- **Visual aids:** Glasses, braille tools, white canes.
- **Hearing aids and communication tools:** Hearing devices, screen readers, speech-to-text software.

For effectiveness, provision of devices must include training, repair services, and environmental adaptations. Affordable and accessible assistive technologies help reduce dependence, combat stigma, and enable persons with disabilities to live independently.

GOAL OF CBID IN HEALTH

Protect, respect, and promote the health rights of persons with disabilities and their families.
Ensure persons with disabilities participate in health-related community activities.
Mainstream disability issues into health services at both community and national levels.

ROLE OF CBID IN HEALTH

CBID promotes inclusive health through:

- o **Health promotion and prevention:** Supporting community awareness, disability-friendly health programs, and lifestyle interventions.
- o **Medical care:** Ensuring persons with disabilities access affordable, quality services at all levels of care.
- o **Rehabilitation and habilitation:** Providing support for restoring or developing skills for independent living.
- o **Assistive devices:** Facilitating access, training, repair, and replacement of devices and technologies.
- o **Advocacy:** Working with health workers and institutions to make services responsive and inclusive.

Role of Health Workers in CBID

Health workers are central to CBID and contribute in the following ways:

- o **Community Education and Awareness:** Educating communities on health and disability issues while promoting positive attitudes.
- o **Therapy and Rehabilitation:** Providing interventions to prepare individuals for full participation in community life.
- o **Guidance and Empowerment:** Sharing resources, offering practical advice, and empowering communities to advocate for themselves.
- o **Program Development and Implementation:** Supporting planning and execution of CBID health programs.
- o **Collaboration:** Coordinating with education, social services, and other sectors.
- o **Early Identification and Referral:** Detecting disabilities early and facilitating timely referrals.
- o **Assistive Technology:** Ensuring access, training, and support for use of devices.

EDUCATION COMPONENT

Inclusive Education and CBID are strategies that address the issues and challenges affecting persons with disabilities and their families. They have been developed to challenge the exclusionary policies and practices, which have predominated in previous decades. They recognize the value of individuals with disabilities and challenge professional and community attitudes.

The main aim of Inclusive Education as a component aim is to include children with disabilities in mainstream education, overcoming barriers to learning and participation in community educational settings. This is related to Community Based Rehabilitation in that individuals with disabilities get equal access to services that are available to other people in the community, such as education and employment. Education empowers and strengthens all people in the community.

The main barriers to inclusive education include attitudinal barriers, poverty lack of teacher training rigid curriculum insufficient segregation practices, support services, marginalization and discrimination. The sustainable development goal 4 states there is need for persons with disabilities to acquire equitable, quality education and promote lifelong learning opportunities. CBID is crucial in the education sector so as to meet all the diverse learning needs for all learners. Community workers and teachers need appropriate training, so that they know how to support and include persons with disabilities within the services that they offer.

Through CBID and Inclusive Education, persons with disabilities are empowered to change their life chances and obtain the means to participate more fully in their communities. This is why CBID and inclusive education are inter-related and co-exist in some communities to complement each other. For instance, CBID workers may identify children with disabilities whom they refer to ordinary schools. This is a first and vital step in the beginning of the inclusive education process and needs to be met with acceptance and interest by the teachers.

KEY CONCEPTS OF CBID AND EDUCATION

Inclusive education: is a process of addressing and responding to the diversity of needs of all learners through increasing meaningful participation in learning, cultures and communities, and reducing exclusion within and from education.

Education Assessment: is a wide variety of methods or tools used to evaluate, measure and document the academic readiness, learning progress, skill acquisition or educational needs of learners and trainees.

Resource Centers: are set up for the educational assessment of children and their placement to appropriate education services.

Extra-curricular activities: are a range of activities organized outside of the regular school day.

Home Based Education: A strategy that employs both community-based rehabilitation and provision of education from neighborhood schools to persons with disabilities in preparation to attend or to transition to school-based learning and to persons with severe multiple disabilities who would otherwise not attend school and thus receive education within their homes

Inclusion: Philosophy which focuses on the process of adjusting home, school, and society so that all the individuals, regardless of their differences, can have the opportunity to interact, play, learn, work and experience the feeling of belonging, and experiment to develop in accordance with their potentials and difficulties.

Resource Room: A separate, remedial classroom in an inclusive school where learners and trainees with disabilities are given occasional direct, specialized instruction and academic remediation and assistance as individuals or in groups.

Meaningful participation: means necessary and appropriate modification and adjustments not imposing a disproportionate or undue burden where needed in a particular case to ensure persons with disabilities exercise and enjoy on an equal basis fundamental rights and freedoms.

Mainstreaming: is a process of supporting programmes through systematic integration of priorities and requirements of persons with disabilities into new and existing legislation, standards, policies and strategies to ensure their participation in all sectors.

GOAL OF CBID IN EDUCATION

Persons with disabilities access education and lifelong learning, leading to fulfilment of potential, a sense of dignity and self-worth, and effective participation in society.

The expected outcomes for Education in relation to Community Based Rehabilitation.

- o All persons with disabilities have access to learning and resources that meet their needs and respect their rights.
- o Local schools admit all children, including children with disabilities, so they can learn and play alongside their peers.
- o Local schools are accessible and have reasonable accommodation through a flexible curriculum and well trained teachers on disability
- o Persons with disabilities are involved in education as role-models, decision-makers and contributors.
- o Home environments that encourage and support learning.
- o Communities are aware that persons with disabilities can learn, and provide support and encouragement.
- o There is good collaboration between the health, education, social and other sectors.

There is systemic advocacy at all levels to make national policies comprehensive to facilitate inclusive education.

ROLE OF CBID IN EDUCATION

The role of CBID is to work with the education sector to help make education inclusive at all levels, and to facilitate access to education and lifelong learning for persons with disabilities.

FORMS OF EDUCATION

Formal Education is an organized and regulated system of education, which follows a curriculum and awards recognized academic certificates. The levels include early childhood care and education, primary education, secondary and higher education.

Early Childhood Care and Education

This is the period from birth to eight years and is the main seed for the inclusive society as children with and without disabilities get to play and learn together. The main goal of this element is that all children with disabilities have the best possible start in life and are supported throughout their development in inclusive learning environments. In CBC, early childhood covers the first two years of a child's education i.e. 2 years pre-primary. The CBC approach is tailored to promote inclusive education environment and flexible learning experience for persons with disabilities.

The desirable outcomes of the CBID program in Early childhood care and education are:

- o All children are developed to their maximum potential in physical, social, language and cognitive skills.
- o Inclusive and welcoming environment for all children to access formal and non-formal early childhood education.
- o Children with disabilities and their caregivers are included in the family setting and community and have appropriate support.

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- Children learn to play together, accept the differences between them and help each other.
 - The impact of impairments is reduced and compensated for.
 - Children with disabilities make a smooth transition to primary schooling together with their peers.

Primary Education

Primary education usually begins from the age of six or seven years old and continues into the early teens. The goal of CBID program is to create a welcoming, inclusive primary education system in local schools within the community. In CBC, primary school covers the first six years of a child's education i.e. grades 1 to 6. At this level the education system seeks to emphasize practical skills, critical thinking and continuous assessment.

The desirable outcomes of the CBID program in primary education are:

- The whole community works towards developing an inclusive primary education system.
- Families create supportive and positive environments for inclusive primary education.
- All children with disabilities complete a quality primary education.
- All the assistive devices, interventions and other necessary assistance are provided so that children with disabilities can participate in inclusive education.
- Proper assessment and identification of issues related to access within the school environment are conducted and addressed.
- Special Needs Education instructors become confident and feel supported by the community.
- inclusive system and approaches for instruction such as curriculum, examination and assessment systems, teaching approaches and extracurricular activities (e.g. sports, music, clubs).
- Appropriate local and specialist resources for education are provided are meaningfully utilized within the education system
- To ensure national policies promote inclusive primary education, needed partnerships are created with stakeholders and proper advocacy at all level is also done.

Secondary and Higher Education

The key role of the CBID program focused on secondary and higher education is to facilitate inclusive education, increase participation of children with disabilities and work with the local authorities and institutions to create an accessible environment and flexible curriculum. In CBC, junior and senior school cover grade 7 to 9 and 10 to 12 respectively. After senior schools students are expected to go to institutions of higher learning for three years. At this level the child is introduced to various career pathways and a deeper understanding of the co-subjects.

The desirable outcomes of the CBID program focused on this element are;

- Increased enrolment, retention and completion in junior, senior schools and higher education by students with disabilities.
- Government grants, scholarships, and other sources of funding are provided to students with disabilities and parents and communities have knowledge and skills to access these kinds of support.
- Communities support lobbying groups and campaigns for equal access to education.

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- Families and communities encourage their children, including children with disabilities, to pursue junior, senior school and higher education.
 - Junior, Senior and higher education programmes are accessible and inclusive in terms of environment, teaching methods, curricula, extracurricular activities (e.g. sports, recreation, music) and assessment and examination systems.
 - Junior and senior schools learn about diversity and inclusion from the experiences of persons with disabilities, and develop skills needed for an inclusive society.
 - Specialist resources and support are used correctly to enhance the inclusion of students with disabilities.
 - Transitions between secondary/higher education programmes and into adult life are well supported and career guidance is accessible and inclusive.

Non-Formal Education

Non-formal education is a strategy that aims at developing knowledge and skills in persons with disabilities and improving their quality of life.

The desirable outcomes of the CBID program with non-formal education as a key element are:

- Self-reliance
- Persons with disabilities participate in non-formal education programmes and learn skills that contribute to their better living conditions.
- Meaningful participation for persons with disabilities in planning and execution of non-formal education.
- Persons with disabilities, family members, organizations for and of persons with disabilities and parents' associations play vital roles in decision-making and implementing non-formal education programmes.

Home Based and Lifelong Education

Home-based Education is training children at home or places other than school. This form of education guarantees personalized tuition. It can employ both the community-based rehabilitation and provision education through teachers going home. The teacher makes regular home visits to impart teaching and learning to the learner and trainee who is at home.

Lifelong Education is a strategy that aims at providing access to persons with disabilities for lifelong learning opportunities and a variety of learning experiences. The opportunity of lifelong learning will help to prevent social exclusion, marginalization, and unemployment of persons with disabilities. i.e, public speaking, financial literacy, entrepreneurship skills.

The desirable outcome of CBID program directed to this element are:

- Favorable environment is created during formal schooling to Youth with disabilities for learning skills and are helped in their transition to the world of work through vocational and careers guidance and peer counseling services.
- Adults with disabilities have access to appropriate, flexible and effective learning opportunities throughout life, for example adult literacy through open schools and higher education through distance learning.

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- Individuals and groups who have special needs, such as youth and adults with significant and multiple disabilities as well as their caregivers and family members, have access to ongoing learning opportunities.
 - Education related to life skills and survival needs including knowledge about reproductive health, sexuality and Human Immunodeficiency Virus (HIV) are provided to youths and adults with disabilities.
 - Each educational institution in the community promotes social inclusion of youth and adults with disabilities by providing lifelong opportunities

LIVELIHOOD COMPONENT

The livelihood component in CBID focuses on enabling persons with disabilities and their families to gain access to employment opportunities, financial resources, and social protection. This component aims to foster self-reliance and improve the overall economic well-being of individuals and communities.

Livelihood is a broader sociological term rooted in the concept of social justice, where culture and identity form its basis and it is dependent upon the landscape and ecology. It is connected with community and property rights indicating dignity, control, empowerment, and sustainability apart from income generation. Livelihoods are dynamic, relying on emerging opportunities that are embedded within constantly changing social and political contexts.

A livelihood comprises the capabilities, resources and activities access required for a means of living. A livelihood is sustainable and can cope with and recover from stress and shocks, maintain or enhance its capabilities and assets, and provide sustainable livelihood opportunities for the next generation, and which contributes benefits to other livelihoods at the local, regional and global levels and in the short- and long-term.

Community-based rehabilitation is therefore important for early learning, skills development, and income-generating activities to improve the lives of persons with disabilities.

How to Include Persons with disabilities in Livelihood Programs

Inclusion of persons with disabilities in livelihood programs is essential to break the cycle of poverty and ensure equal participation in society. This requires integrating them into formal employment, income-generating projects, skills development, and access to loans and financial services. A human rights approach to disability guides this process, ensuring that persons with disabilities benefit fully from all development initiatives.

Key Principles of Inclusion

- 1. Awareness:** Understand disability and its implications for livelihoods.
- 2. Participation:** Actively involve persons with disabilities in designing, implementing, and evaluating programs.
- 3. Accessibility:** Address physical, communication, policy, and attitudinal barriers that limit participation.
- 4. Twin-Track Approach:** Combine disability-specific actions with mainstream development strategies to ensure comprehensive inclusion.

Practical Actions for Awareness

- Identify the number of persons with disabilities in the community using census data, household surveys, or information from Disabled Persons' Organizations (DPOs), Community-Based Rehabilitation (CBR) programs, and inclusive education services.
- Promote the strengths, skills, and contributions of persons with disabilities in livelihood and employment opportunities, highlighting their valuable role in community development.
- Connect with CBR and related programs to improve access to training, financial services, and broader livelihood initiatives.
- Advocate for the rights of persons with disabilities to participate fully in economic life, ensuring policies and programs address their needs.
- Educate communities to challenge stigma and build acceptance of the rights of persons with disabilities to engage in income generation, employment, and financial independence.

GOAL OF CBID IN LIVELIHOOD

To ensure that a person with a disability gains a source of livelihood, access social protection measures to live a dignified life, and contribute economically and socially to the lives of their families and communities.

Persons with disabilities can access self-employment opportunities, social opportunities improving their standard of living to contribute to the well-being of families and communities and they are treated equally.

ROLE OF CBID IN LIVELIHOOD

The role of the CBID is to facilitate by working with relevant stakeholders to ensure access and full participation of persons with disabilities in the social life of their families and communities as well as to acquire skills, livelihood opportunities, and meaningful engagements within the community.

The role of CBID is to encourage and support self-employment by assisting persons with disabilities and their families, either individually or in groups, to access skills development as well as financial and material resources.

The desirable outcome of the CBID program focused on this component are:

- Access to education, skills acquisition, and work opportunities for children with disabilities.
- Access to work opportunities without discrimination in a friendly environment.
- Persons with disabilities have an equal opportunity to work and employment.
- Families of persons with disabilities, especially children and people with severe disabilities, have access to better means of livelihood.
- Recognition of the contribution of persons with disabilities at their workplaces and their employers.
- Persons with disabilities accessing social protection programs.
- All poverty reduction strategies and programs include and benefit persons with disabilities and their families.
- Enactment of 5% of persons with disabilities in every institution whether in private, public sectors.

- Persons with disabilities earn income through their own chosen economic activities as individuals or in groups.
- Mainstream government and nongovernmental small-enterprise development programs change their policies and practices to include persons with disabilities in the training and assistance they provide.
- Persons with disabilities can access business skills through training.
- Persons with disabilities can access financial services such as loans, and credit facilities for their business.
- Persons with disabilities have access to further business skills development opportunities to progress in their economic activities.
- Persons with disabilities are recognized as successful entrepreneurs and productive, contributing members of the community.
- Communities recognize that persons with disabilities are valued members and can make positive contributions to community development.
- Persons with disabilities willing to form and be registered as self-help groups, be registered for free by the department; waiver of registration fee.
- Barriers that exclude persons with disabilities and their families from participation in social roles and activities are challenged and addressed.
- Mainstream providers of vocational and skills training have policies and practices that ensure persons with disabilities have access to the training provided.
- Persons with disabilities have access to support services – vocational guidance, placement services, assistive devices and adapted equipment.
- Persons with disabilities have access to advanced skills development opportunities required to progress in their work.

ELEMENTS OF THE LIVELIHOOD COMPONENT

Self-Employment

Self-employment refers to economic activities owned, operated, and managed by individuals or groups. It is one of the main opportunities for persons with disabilities, especially in low-income countries, to earn a living. These activities can be part-time or full -time, in both rural and urban areas, and in either the formal or informal economy.

The State Department for Social Protection supports self-employment by offering vocational training through 12 Vocational Rehabilitation Centres (Vices) across the country. Training areas include beadwork, hairdressing, electrical engineering, ICT, shoemaking, leatherwork, and other trades.

CBID programs also play a vital role in helping persons with disabilities start or expand small businesses and income-generating activities.

Types of Enterprises (WHO, 2019)

- **Income-generating activities:** Small-scale ventures, often traditional or seasonal, that supplement or provide the main source of income (e.g., farming, poultry raising, crafts).
- **Small and medium enterprises (SMEs):** Larger businesses that employ more than one person and serve as the principal income source, usually in production or service delivery.
- **Self-help groups:** Groups formed to pool savings, provide mutual support, and sometimes run joint businesses to reduce risk and increase scale.

Wage Employment

Wage employment refers to paid jobs under contract with an organization or employer. While this is more common in the formal economy, it can also occur in informal work.

Persons with disabilities often face barriers in accessing decent wage employment, but progress is being made as more businesses adopt inclusive hiring practices. CBID programs help reduce barriers and support equal access to decent employment opportunities.

Financial Services

Persons with disabilities require the same access to financial services as everyone else to start, grow, and sustain livelihoods. These services include:

- Savings and loans from Saccos, banks, micro-finance institutions, mobile loan platforms, and self-help groups (chamas).
- Grants and social funds for small businesses.
- Financial literacy programs.
- Waivers and tax exemptions encourage economic participation.

Social Protection

Social protection programs provide a safety net against poverty, income loss, and vulnerability due to illness, disability, or old age. Persons with disabilities have equal rights to these programs, which include:

- Cash Transfer for Orphans and Vulnerable Children (CT-OVC)
- Cash Transfer for Older Persons (OPCT)
- Cash Transfer for Persons with Severe Disabilities (PWSD-CT)
- Hunger Safety Net Program (HSNP)

Skills Development

The goal of skills development is to ensure persons with disabilities acquire the knowledge, skills, and attitudes necessary for dignified work and sustainable livelihoods. CBID supports this by facilitating access to training, vocational guidance, and placement services, while advocating for inclusive policies in mainstream vocational institutions.

The expected outcomes on skills development include:

- Youth and adults with disabilities acquire marketable skills and access decent work (wage or self-employment).
- Women and girls have equal opportunities alongside men and boys.
- Inclusive vocational training institutions adopt disability-friendly practices.
- Persons with disabilities access advanced training opportunities for career progression.

Types of Skills

- **Foundation skills:** Literacy, numeracy, problem-solving, and learning ability.
- **Technical and vocational skills:** Carpentry, tailoring, weaving, welding, shoemaking, ICT, and others.
- **Professional skills:** Advanced training such as engineering, physiotherapy, medicine, and computer technology.
- **Business/entrepreneurial skills:** Planning, financial management, marketing, and risk assessment.
- **Core life skills:** Communication, teamwork, customer relations, self-confidence, and decision-making.

Ways to Acquire Skills

- Self-learning and family-based training.
- Basic education and community-based apprenticeships.
- Vocational training centres (VRCs).
- On-the-job training, internships, or volunteering.
- College/university education.
- Online learning and digital skills programs.

SOCIAL COMPONENT

The social component in CBID focuses on ensuring that individuals with disabilities can fully participate in their communities' social lives. This involves promoting access to social opportunities, challenging discrimination, and encouraging positive social change. This component entails living together or enjoying life in communities or organized groups.

Social organisms including humans, live collectively in interacting with populations. It also refers to seeking or enjoying the companionship of others in a community rather than in isolation. The chance to participate in social activities has a great impact on the individual's self-esteem, quality of life and their own social status.

Persons with disabilities face many barriers and fewer opportunities while participating in social activities. In this component, the CBID program is to ensure that persons with disabilities have meaningful social roles and responsibilities in the community and family and that they are viewed as equal members of society.

KEY CONCEPTS OF THE SOCIAL COMPONENT

Social Inclusion: It is defined in disability as being accepted as individuals, having personal relationships with family, friends, and others as well as being involved in recreation, leisure, and other activities. Social inclusion can also be defined as greater participation in community-based activities and a broader social context of life. Social inclusion can also be referred to as a process of improving the terms of participation in society, particularly for disadvantaged people.

Social Roles: These are positions people hold in society that are associated with certain responsibilities and activities.

Culture and Arts: These are programs, services, and activities that play a key role in a community's social and economic well-being.

Personal Assistance: Support a person with disability needs from another person for everyday living. The important aspect of personal assistance is that the person chooses who will assist and how that assistance will be given.

Recreation and Leisure: Activities where the primary purpose is participation, with the related goals of improved physical fitness, fun, and social involvement often prominent.

Justice: Is an essential value in all societies and is governed by the application of the law. This principle aims to ensure that actions are taken truthfully and that each person receives what is due, acting objectively and fairly.

Relationships, Marriage, and Family: -Supporting individuals with disabilities in forming and maintaining meaningful relationships and family connections.

GOAL OF CBID IN SOCIAL

Making sure that persons with disabilities and their families can make their own decisions, meet their basic needs, and change their lives positively as well as improve their communities.

ROLE OF CBID IN SOCIAL

The role of the CBID is to collaborate with all relevant stakeholders to ensure the full participation of people with disabilities in the social life of their families and communities. CBID programmes can provide support and assistance to people with disabilities to enable them to access social opportunities and can challenge stigma and discrimination to bring about positive social change.

The desirable outcome of the CBID program focused on this component are:

- Persons with disabilities are valued as members of their families and have a variety of social roles and responsibilities.
- Persons with disabilities and their families are encouraged and supported to contribute their skills and resources to the development of their communities.
- Communities recognize that people with disabilities are valued members and can make positive contributions to the community.
- Barriers that exclude people with disabilities and their families from participating in social roles and activities are challenged and addressed.
- Local government authorities respond to the needs of people with disabilities and their family members and provide effective social support and services where required.

Addressing Barriers and Ensuring Inclusion

- Barriers faced by persons with disabilities and their families while participating in social roles are identified and addressed.
- Persons with disabilities have access to both formal mechanisms of justice (such as courts) and informal mechanisms (such as village elders or leaders) when their rights are violated.
- CBID programmes are established to support persons with disabilities in accessing justice and protecting their rights.

Promoting Dignity, Equality, and Social Roles

- Persons with disabilities are recognized as valued members of society with various social roles.
- Communities acknowledge that persons with disabilities can make positive contributions and actively value their participation.
- There is a reduction in stigma and discrimination within communities.
- Awareness and acceptance are strengthened that persons with disabilities can form meaningful relationships, marry, and raise families within the ordinary community setup.

Justice

The role of CBID in justice is to promote awareness of the rights of persons with disabilities and provide support to persons with disabilities and their family members to access justice where they face discrimination and exclusion.

Rights holders and duty bearers: Understanding the relationship between rights holders and duty bearers is an important concept when considering the rights of persons with disabilities and enabling their access to justice.

Rights holders: Persons with disabilities are rights holders, i.e. they have both entitlements and responsibilities. As rights holders they are entitled to, for example, health, education, livelihood opportunities, land, housing, and political participation. These, as well as other entitlements, are all outlined in the Convention on the Rights of Persons with Disabilities.

Duty bearers: When a person has a right, someone else has a duty to respect, protect and fulfil that right. Duty bearers include both state (government) and non-state (e.g. non-governmental organizations, religious leaders, parents) actors at national and local levels. The duties of states are outlined in the Convention on the Rights of Persons with Disabilities.

The desired outcomes of CBID in the element of Justice include:

- CBR programmes are able to support people with disabilities to access justice when the need arises.
- People with disabilities are aware of their rights and the options and procedures for accessing justice.
- Relevant stakeholders in the justice sector are sensitive to the needs of people with disabilities and do not engage in discriminatory practices.
- People with disabilities are able to access informal mechanisms of justice when their rights are violated.
- People with disabilities are able to access formal mechanisms of justice when their rights are violated.

Cultural, Spiritual, and Social Participation

- Persons with disabilities and their families actively participate in cultural and artistic activities.
- Spiritual and religious leaders ensure the inclusion of persons with disabilities in worship and faith-based activities.

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- Recreation, sports, and leisure programs are developed with adaptations to equipment and facilities to meet the needs of persons with disabilities, while recognizing their rights to equal participation.

How to Include Persons with disabilities in Livelihood Programs

It is essential to ensure the inclusion of persons with disabilities in all livelihood approaches, including formal employment, income generation projects, skills development, and access to loans and financial services. Inclusion should be guided by a human-rights approach to disability, ensuring equal participation in all development programs and sectors.

Key Principles of Inclusion

- 1. Awareness:** Understand disability and its implications for livelihoods.
- 2. Participation:** Actively involve persons with disabilities in the design, implementation, and evaluation of programs.
- 3. Accessibility:** Address barriers in the physical, communication, policy, and attitudinal environment.
- 4. Twin-Track Approach:** Combine disability-specific interventions with mainstream development programs.

Practical Steps for Awareness

- Identify the number of persons with disabilities in the community using census data, household surveys, or partnerships with Disabled Persons' Organizations (DPOs), Community-Based Rehabilitation (CBR) programs, and inclusive education services.
- Promote the strengths, skills, and contributions of persons with disabilities in livelihood and employment opportunities.
- Link with CBR and other programs to increase access to training, financial services, and broader development initiatives.
- Advocate and educate the community on the rights of persons with disabilities to access livelihood opportunities, emphasizing that breaking the cycle of poverty and disability requires access to income-generating activities.

EMPOWERMENT COMPONENT

Empowerment is a social process where persons with disabilities, their families, and the community know and access their rights, act on issues they deem important, and contribute meaningfully to society.

The empowerment process begins when persons with disabilities and their communities recognize that they can change their situation and begin to do so. Awareness and capacity-building are building blocks in empowerment that result in participation, decision-making, and consequently action for change.

KEY CONCEPTS OF THE EMPOWERMENT COMPONENT

Capacity-building: Persons with disabilities need a range of skills and knowledge to enable them to participate and contribute meaningfully to their families and communities. Gaining skills and knowledge can lead to increased confidence and self-esteem, which is an important part of the empowerment process.

Participation: Being a contributor brings social recognition and is important in the empowerment process. People with disabilities can participate and make positive contributions at many diverse levels.

Self-Advocacy: This refers to Persons with disabilities having a voice of their own. It means knowing about their rights and responsibilities, speaking up for their rights, and being able to make choices and decisions about their lives.

Political participation: Promoting the participation of persons with disabilities in politics is an important approach to empowerment.

GOAL OF CBID IN EMPOWERMENT

Persons with disabilities make their own decisions and take responsibility for changing their lives and improving their communities.

ROLE OF CBID IN EMPOWERMENT

CBID contributes to the empowerment process by promoting, supporting, and facilitating active involvement of persons with disabilities and their families in issues that affect their lives.

The desirable outcome of the CBID program focused on this component are:

- Persons with disabilities are active participants and contributors in their families and communities.
- Increased community awareness on capabilities and possibilities of Persons with disabilities
- Access and availability to information improves, leading to a knowledgeable community.
- Persons with disabilities can make informed choices and decisions.
- Barriers faced by Persons with disabilities are reduced or eliminated.
- Persons with disabilities are accepted in the communities and their potential is recognized.
- Persons with disabilities and their families can access development benefits and services in their communities.
- People with disabilities and their family members come together, form their groups and organizations, and work towards addressing their common problems.
- OPDs promote CBID, and members become involved in the planning and implementation of CBID programmes.
- CBID programmes and OPDs work together to plan, implement, and monitor new or existing CBID programmes.
- Persons with disabilities are valued as members of their families and have a variety of social roles and responsibilities.
- Persons with disabilities and their families are encouraged and supported to contribute their skills and resources to the development of their communities.

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- Communities recognize that people with disabilities are valued members and can make positive contributions to the community.
 - Barriers that exclude people with disabilities and their families from participating in social roles and activities are challenged and addressed.
 - Local government authorities respond to the needs of people with disabilities and their family members and provide effective social support and services where required.

CHAPTER 4

STATE-LED PROGRAMMES AND KEY ROLES

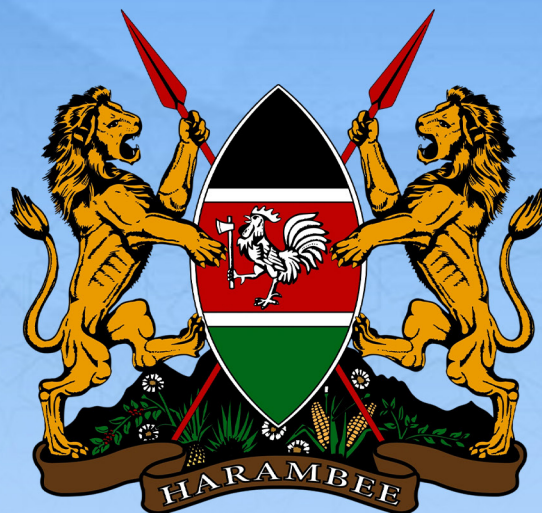
INSTITUTION	KEY ROLES
State Department for Social Protection	• Cash Transfer Programme for persons with severe disabilities • Registration of community groups • Vocational rehabilitation training for economic self-reliance
State Department for Labour	• Employment skills training • Certification and industrial attachment through National Industrial Training Authority
Ministry of Health	• Medical assessments for disability certification • Physiotherapy and occupational therapy • Habilitation services for children • Counseling and specialized care • Sunscreens for persons with albinism • Comprehensive health coverage under UHC and Social Health Insurance
Ministry of Education	• Inclusive education policy implementation • Teacher training in Special Needs Education (SNE) • Capitation grants for learners with disabilities • Accessible learning materials and exam accommodations • Support services like learning assistants
Kenya Revenue Authority (KRA)	• Tax exemptions for registered persons with disabilities and employers • Tax incentives for employers
National Council for Persons with Disabilities (NCPWD)	• Registration of persons with disabilities • Education assistance (bursaries, scholarships) • Assistive devices and tools of trade • Seed capital and LPO financing • Enforce accessibility and support employment

National Fund for the Disabled of Kenya (NFDK)	• Assistive devices (wheelchairs, hearing aids) • Start-up toolkits for income-generating activities • Grants for community projects • Construction and refurbishment of facilities
Kenya Institute for Special Education (KISE)	• Teacher training in special needs and inclusive education • In-service courses for professionals • Educational and psychological assessment • Orientation and mobility training • Pre-school department for early intervention • Production and repair of specialized learning materials
Kenya Institute of Curriculum Development (KICD)	• Development of specialized curriculum • Adaptation of inclusive learning resources • Teacher training collaboration • Quality assurance and approval • Dissemination of information
County Governments	• Provision of health and rehabilitation services • Resource allocation • Disability information coordination • Affirmative action in procurement • ECDE services • Accessible service delivery and enforcement • Microfinance initiatives • Cash transfers for persons with severe disabilities

NON-STATE ACTORS AND DEVELOPMENT PARTNERS

The following non-state actors are active in different aspects of CBID. They support government efforts in the delivery of CBID. These include.

CATEGORY	ORGANIZATIONS / PARTNERS
Core Organizations	Christian Blind Mission, SightSavers International, Light for the World, Motivation Trust, Humanity Inclusion, The Action Foundation, United Disabled Persons of Kenya, Association for the Physically Disabled of Kenya (APDK), Sense International
Disability Societies	Kenya Society for Deaf Children and Kenya Society for the Blind
Organizations of Persons with Disabilities (OPDs)	KAIH, DTSK, Down Syndrome Society of Kenya, Andy Speaks, SHAK
Other Organizations	Bethany Kids (Kijabe), AIC Cure International
Other Partners	Private Sector Actors, Faith-Based Organizations (FBOs)
Academic Institutions	Universities and Colleges (KU, JKUAT, MKU, UoN)
UN Agencies	UNICEF, ILO, UNFPA, WFP, UN Women
Financial Institutions	KCB and other financial actors



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